Office of Professional Licensure Review
UTAH DEPARTMENT OF COMMERCE

2025 Periodic Review

Athletic Training

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Executive Summary

Background

Athletic trainers (ATs) are healthcare professionals trained at a graduate level that collaborate with physicians to provide preventative services, emergency care, therapeutic intervention, and rehabilitation of injuries and medical conditions. ATs perform their work within the context of sports and physical activity. They engage in proactive education efforts and physical training to reduce the occurrence of serious injuries and help rehabilitate injuries and disabilities when they occur. ATs are also frontline responders during athletic events, handling acute injuries, managing on-the-spot emergencies, and determining when an athlete may return to play.

Every state except California requires ATs to be licensed. Licensure requirements, including graduation from an accredited athletic training program and passing an examination, are relatively standard throughout the U.S. Although the requirements to be licensed are largely standardized, states differ in their approach to physician collaboration and AT scope of practice. Many states require physician direction for all work an AT performs, while some states specify when physician direction is necessary, and a small minority do not require physician collaboration at all. Nearly every state restricts ATs to treating athletic injuries incurred by athletes, although the definitions of injury and athlete, and thus the scope of practice, vary state-to-state.

Regulatory Model Recommendation: *Continue to license ATs*

- ATs perform some duties, such as determining whether to remove an athlete from the field, that may have serious and potentially deadly consequences if performed poorly.
- Despite relatively high levels of employer oversight, many ATs work with minors, whom they often must physically touch, sometimes in private settings. This has implications for the risk of ethical and sexual misconduct.

Recommended Regulatory Adjustments: *None*

- Given indications of safe practice and few accessibility issues, OPLR does not recommend significant adjustments to the current licensing model.

Additional Recommendations:

1) *Clarify that an AT must collaborate with a physician **only** when rehabilitating an injury that is 1) not responding, 2) beyond the AT's scope, or 3) a head or brain injury*
    - Requiring physician direction on all injury rehabilitation ignores the extensive education and training that ATs receive during their master's degree.
    - Clarification may also increase safety and efficacy by requiring ATs to contact physicians proactively when initial treatment is not effective.
2) *Explicitly allow ATs to treat work-related injuries*
    - ATs commonly work within occupational, industrial, and other non-traditional settings and many other states explicitly allow this practice

Context

Consistent with its legislative mandate,[1] the Office of Professional Licensure Review (OPLR) reviewed Utah's licensing laws for athletic trainers. The review evaluated how well current regulations:

1. Protect the public from present and consequential physical and financial harm
2. Balance public and practitioner access to the occupation
3. Limit the economic impact of regulation on consumers, practitioners and the state[2]

OPLR's research for this review included analysis of Utah's current laws and rules, licensing and complaint data from the Utah Division of Professional Licensing (DOPL), licensee survey results, academic literature, health workforce data, as well as laws and policies in other states. OPLR also conducted interviews with athletic trainers, employers, faculty, and other subject matter experts. See Appendix 1.1 for more information.

Background

Profession Overview

Athletic trainers (ATs) are healthcare professionals that collaborate with physicians to provide preventative services, emergency care, therapeutic intervention, and rehabilitation of injuries and medical conditions. ATs perform their work solely within the context of sports and physical activity, differentiating them from related professions, like physical therapy, which treat injury and disease among all population groups. Despite their specialization, ATs perform many job duties; they engage in proactive education efforts and physical training to help reduce the occurrence of serious injuries, and help rehabilitate injuries and disabilities when they occur.[3] ATs are also frontline responders during athletic events, handling acute injuries, managing on-the-spot emergencies, and determining when an athlete may return to play.

The American Medical Association recognizes athletic training as an allied health care profession due to the advanced education and training.[4] This distinguishes them from personal trainers, who may receive no formal education and whose primary aim is to provide guidance on exercise and fitness.

ATs work in a variety of settings with physically active individuals of all ages. ATs most commonly work in a "traditional" setting with athletes who participate in organized sports at the high-school, collegiate, and professional levels.[5] "Non-traditional" settings have become more common over time. ATs increasingly provide care for individuals in industrial and corporate

[1] UCA 13-1b-203(2)
[2] UCA 13-1b
[3] NATA What is Athletic Training?
[4] Ibid
[5] According to the 2022-2023 BOC Certification Maintenance Data, ~25% work in high-schools, ~16% work in universities, ~4% work in professional sports and the performing arts, and ~1% work in youth sports.

settings, and alongside physicians and physical therapists in hospitals and sports medicine and rehabilitation clinics.[6] ATs are not typically independent providers; they almost always work in collaboration with or under the direction of a licensed physician.

Profession in Utah

To legally work as an AT in Utah, an individual must attain a license through (DOPL). This requires:

1. Obtaining a bachelor's or advanced degree that is accredited by or meets the athletic training curriculum standards established by the Commission on Accreditation of Athletic Training Education (CAATE);[7]
2. Passing the certification exam administered by the Board of Certification (BOC); and
3. Holding the Athletic Trainer Certification (ATC) credential from the BOC in good standing.[8,9]

Licensed ATs must renew their license every two years, although DOPL does not require ATs to meet any continuing education or certification requirements.

There were 846 licensed ATs in Utah as of the beginning of 2025.

Approaches in Other Jurisdictions

Every state in the U.S., with the exception of California, licenses ATs.[10] Rather than license ATs, California has adopted title protection laws which require individuals to meet certain requirements in order to use the title of athletic trainer, but not to perform athletic training services.[11,12] Otherwise, requirements for licensure are functionally identical across the U.S., as nearly every state (except Texas) requires ATs to pass the BOC exam. To sit for the exam, an individual must have graduated from a CAATE accredited athletic training program. Therefore, all of these states require applicants to meet the education standard set by CAATE regardless of the educational requirement set in statute. Even though many states, including Utah, allow an

[6] According to the 2022-2023 BOC Certification Maintenance Data, ~18% work in hospital and clinic settings, ~4% work in industrial and corporate settings, and ~2% work in the military/government/law enforcement. The last ~30% work in settings that do not require AT licensure (i.e. administrator, educator, student) or are unspecified.
[7] Although Utah statute allows applicants to hold a bachelor's degree, CAATE no longer accredits any bachelor's level programs.
[8] UCA 58-40a
[9] To achieve the ATC credential, an applicant must: 1) achieve emergency cardiac care (ECC) certification, 2) complete a CAATE accredited program, and 3) pass the BOC exam. See the BOC's Certification Exam Candidate Handbook for more information.
[10] OPLR Policy Scan
[11] California Assembly Bill 796 (2023-2024)
[12] It is unclear how ATs in California legally work without engaging in unlicensed medical practice, given that ATs are healthcare providers and much of their scope of practice overlaps with existing licenses, such as physicians and physical therapists. OPLR reached out to athletic trainer industry organizations and regulators in California but received no clarification.

individual to hold a bachelor's degree, CAATE now only accredits master's level AT programs.[13] As a result, a master's degree is the de-facto educational requirement for licensure.

Texas diverges from this model through the establishment of their own athletic trainer exam, which allows them to accept multiple pathways to licensure. Besides graduation from a CAATE accredited AT program, Texas accepts the following: 1) attainment of a bachelor's degree that meets certain criteria and completion of an 1,800 hour apprenticeship and 2) a physical therapy license and completion of a 720-hour apprenticeship.[14]

Although the requirements to be licensed are largely standardized, states differ widely in their approach to physician collaboration and AT scope of practice.

Most states, including Utah, require ATs to work in collaboration with, or under the direction of, a licensed physician. This is typically defined as following written and/or verbal protocols or athletic training service plans established or approved by a physician. Many states require physician direction for all work an AT performs, while some states, such as Utah and Oregon, specify when physician direction is necessary.[15] Montana and Vermont are the only states that do not include provisions requiring physician direction or collaboration.

Nearly every state restricts ATs to treating athletic injuries incurred by athletes, although the definitions of injury and athlete, and thus the scope of practice, vary state-to-state. A minority of states, such as Arkansas and Massachusetts, restrict ATs to only treating injuries incurred by individuals participating in organized or recreational sports.[16] At least 11 states either explicitly allow ATs to also treat individuals participating in physically demanding employment activities or define their terms broadly enough to encompass this population. Many states, like Utah and Alabama, narrowly define "athlete" and "athletic injury", but allow ATs to treat a broader population as long as a directing physician has given authorization or referral.[17]

A few states also explicitly allow ATs to perform specific tasks, such as dry needling.[18] Kentucky has the most expansive scope for ATs. They may draw blood, repair or close superficial lacerations, and provide IV medication injections, among some other invasive procedures.[19]

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[13] Joint Statement from the AT Strategic Alliance (consisting of the following organizations: BOC, CAATE, NATA and NATA Foundation)
[14] By statute, Oklahoma accepts the same alternative education requirements that Texas does. However, Oklahoma requires applicants to take the BOC exam, thereby restricting individuals from pursuing an alternative path to licensure.
[15] Utah specifies that all reconditioning and rehabilitation of injuries must be done under direction of a physician, whereas Oregon requires physician collaboration only in the following circumstances: 1) the athletic injury is beyond the AT's scope of practice or expertise, or 2) the athletic injury is not responding to treatment
[16] A few more states, like Colorado, employ vague language like "exercise" or "physical fitness activities", making it unclear as to how narrowly they define athlete or athletic injury.
[17] Many of these states allow licensed physical therapists or other advanced healthcare practitioners to refer or authorize as well.
[18] 11 states allow ATs to engage in dry needling. See: BOC (2025) Dry Needling State Regulation Updates
[19] 201 KAR 9:290

Regulatory Model Assessment & Recommendation

The Framework

In an effort to standardize how appropriate regulatory models are determined for each profession (e.g. license, registry, no regulation, etc.), OPLR developed a framework which incorporates its statutory review criteria.[20] Appropriate models are determined principally by an evaluation of the potential for harm and related factors that may aggravate or mitigate the potential for harm. These factors include the availability of consumer choice, vulnerability of patients, and independence of practice. See Appendix 2.1 for potential regulatory models and the factors in OPLR's framework.

Recommendation

OPLR determined that licensing is the most appropriate form of regulation for athletic training. Significant considerations for this recommendation include: the potential for harm inherent in the practice of athletic training, downstream implications, patient vulnerability and the frequency of physical touch, and the lack of patient choice.[21]

Potential for harm

The practice of athletic training involves providing emergency medical care, determining when an injury is too severe to allow an individual to return to their activity, and performing manual therapies to rehabilitate injury and disability. Poor or untrained performance of this scope may exacerbate an individual's current injury or disability or cause new, potentially very serious injury. Allowing a severely injured athlete (e.g., one showing signs of a TBI, heart condition, heat stroke, internal bleeding, etc.) to return to their sport can, and has, caused serious injury or death.[22] The risk of causing very serious injury is recognized by O*Net's Consequence of Error, which rates professions on a 100-point scale based on the seriousness of potential harm. O*Net gives ATs a score of 72 out of 100, which is in the range of many other licensed health care practitioners.

Although deadly injuries are uncommon, allowing an individual with a non life-threatening injury to return to their activity before they are healed, or improperly treating an injury, may cause further harm or reinjury. This may have serious downstream implications, affecting an

[20] Among other criteria, OPLR is required to evaluate "whether the regulation of the occupation is necessary to address a present, recognizable, and significant harm to the health, safety, or financial welfare of the public" and consider "potentially less burdensome alternatives to the… existing regulation". UCA 13-1b-302

[21] This is not a comprehensive list of all factors. Please see Appendix 2.2 for more information on OPLR's analysis of the potential for harm related to athletic training.

[22] See: Parsons et al. (2019) Preventing Catastrophic Injury and Death in Collegiate Athletes: Interassociation Recommendations Endorsed by 13 Medical and Sports Medicine Organisations; Douglas et. al. (2012) National Athletic Trainers' Association Position Statement: Preventing Sudden Death in Sports; Mueller (2001) Catastrophic Head Injuries in High School and Collegiate Sports; Boden et. al. (2007) Catastrophic Head Injuries in High School and College Football Players

individual's quality of life and/or necessitating further, potentially invasive, corrective medical interventions from other healthcare providers.

The potential for harm is further exacerbated by the setting in which many ATs work. A plurality of ATs work with minors, whom they often must physically touch, sometimes in private settings. This increases the risk of ethical and sexual misconduct.

Related harm factors

ATs practice with less independence than healthcare professionals in related fields (e.g., physical therapy, occupational therapy) and are often employed by larger institutions, both of which ensure a general level of oversight. ATs must collaborate with physicians, although they do exercise independent judgement when making on-the-spot decisions, such as when deciding whether to allow an athlete to continue playing. Still, employers are incentivized to hire competent workers and can fire or discipline those who harm individuals. The BOC, as the sole certifying body, may also discipline ATs and revoke their certification for unethical or unprofessional behavior, although the BOC lacks enforcement power.[23] This level of employer and private oversight helps mitigate some of the harm posed by incompetent practice.

There is little consumer oversight of AT services because ATs are typically employees within large organizations, and individuals are assigned an AT with no input. This can increase the risk of harm in cases where employers fail to provide sufficient oversight, as there is no way for individuals to vet and avoid potentially unsafe or underqualified practitioners.

Given the high degree of oversight overall, OPLR initially considered recommending exemption as a regulatory model, where the practice of athletic training would be exempted from the Utah Medical Practice Act as long as an individual met certain requirements. The exemption model is solely reactive–DOPL would no longer license ATs or have the ability to discipline individuals for anything beyond unlicensed practice of medicine. DOPL would only be able to take action if a complaint were made against an individual practicing as an AT that did not meet the requirements for exemption.

Although exemption is an acceptable model, OPLR determined that licensing better protects the public against the potentially severe harm from incompetent practice or sexual and ethical misconduct towards minors, the population ATs work with most frequently.

Regulatory Model Adjustments & Recommendations

Framework

After determining an appropriate regulatory model, OPLR's framework also evaluates whether adjustments should be made within a recommended model to address any material and existing safety and access issues affecting the Utah public and practitioners. Regulatory model

[23] Although BOC lacks enforcement power, employers can use their data to further screen applicants for AT positions.

adjustments may include changing entry qualifications, the scope of practice, unprofessional or unlawful conduct, and/or supervision and independence provisions (see Appendix 3.1).

Safety Issues

OPLR found no evidence that licensed ATs in Utah are causing severe or systemic harm to the public. OPLR analyzed DOPL's complaint data and found that ATs have among the lowest rates of substantiated complaints of all professions reviewed in 2025, receiving only 7 substantiated complaints between 2017-2022.[24] The majority of these complaints were related to scope of practice or ethics violations and did not result in client harm,[25] except in one case of severe sexual misconduct.

Data from the BOC's Disciplinary Action Exchange illustrates a similar trend. The BOC has suspended only 4 ATs from Utah, 3 of which were due to failing CE/renewal requirement audits and unrelated to client safety.[26] The last was related to sexual misconduct, but it is unclear if the incident occurred while the individual was acting as an AT.

Access Issues

OPLR found mixed evidence regarding the accessibility of AT services. Quantitative data suggests Utah has an adequate supply, though reliable information is limited. The U.S. Health Resources and Services Administration (HRSA) does not collect data on ATs, and the Utah Department of Workforce Services (DWS) and U.S. Bureau of Labor Statistics (BLS) provide little reliable information relevant to the Utah workforce. National estimates from the BOC suggest that 1.41% of BOC certified ATs work in Utah,[27] which is higher than the national share of adults working in Utah.[28] The number of licensed ATs in Utah grew ~5.7% on an annualized basis between 2014-2025,[29] far above the growth in Utah's population.[30] Utah also has four

[24] DOPL Complaint Analysis. Between 2017-2022, ATs had a rate of 0.62 substantiated complaints per 100 licensees. See Appendix 3.2 for more information. OPLR notes that comparisons of complaint rates across professions should not be viewed as determinative, given differences in the scopes of practice and complexity of services between professions.
[25] One scope of practice complaint alleged that they witnessed an AT potentially cause client harm, but this was not verified by investigators.
[26] The BOC's Disciplinary Action Exchange contains public disciplinary decisions made by the BOC or by state regulatory agencies. Data goes back to 2002 and includes all individuals still currently under discipline from that time. For more information, see: https://bocatc.org/state-regulation/#disciplinary-action-exchange
[27] BOC (2024) Certified AT Demographics
[28] The civilian labor force in Utah represents 1.08% of the national civilian labor force. Estimates from the BLS labor force data last updated as of September, 2025. See: BLS (2025) Civilian Labor Force and Unemployment by State, seasonally adjusted
[29] DOPL Historic Licensing Data. See Appendix 3.3 for more information on OPLR's use of this data
[30] In the same time period, Utah's population grew by ~1.8% on an annualized basis. OPLR used the annual Utah population estimates from the IBIS Query Builder using Kem C. Gardner Policy Institute Population Estimates. See: https://ibis.utah.gov/ibisph-view/query/builder/pop/PopKemG/Count.html

accredited AT programs, which graduate a combined 53 students annually and provide the state with a modest yet continuous supply of ATs.[31,32]

At the same time, interviews with relevant industry stakeholders indicated that the supply of ATs may not adequately meet demand in Utah, especially in the secondary school setting. Educators expressed concern at the perceived movement of ATs away from "traditional", school/athletic settings, as the number of AT graduates entering "non-traditional" settings has grown.[33] Despite this, many ATs work in "traditional" settings, and the largest employer of ATs in Utah sees more growth in these settings than in healthcare clinics.[34] Although traditional AT jobs are still being filled, stakeholders indicate the number of applicants has decreased and the time to fill jobs has increased.[35] This may be due to a variety of factors, such as lower relative compensation in these settings despite increasing educational requirements, long irregular hours, and the advance of part-time rather than full-time positions.

Individuals looking to enter the AT profession do face moderate barriers to entry that have grown over the past decade. Due to changes made by CAATE, the entry-level degree recently moved from a bachelor's to a master's, imposing greater costs on individuals looking to enter the field.[36] Program costs in Utah range from ~$18k to ~$90k.[37] Education debt could be difficult to repay, as ATs have a median salary of $63,190 in Utah,[38] which is higher than the nationwide estimate for ATs ($60,250)[39] but significantly lower than the nationwide median for master's-level educated individuals (~$92,000).[40]

Recommendation

OPLR does not currently recommend any significant adjustments to the current licensing model for athletic training due to the lack of pressing concerns related to safety and little evidence suggesting severe accessibility issues. Please see "Other Considerations" for OPLR's recommendation should access become significantly more constrained.

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[31] Information sourced from CAATE's Program Information and Outcomes data. CAATE requires all accredited programs to publish a 3 year aggregated estimate of the number of graduates. OPLR combined estimates across all Utah programs and annualized it. Therefore, the annual estimates may contain inaccuracies, but the aggregate estimate (158) should not.
[32] The number of graduates going into the Utah AT workforce is likely lower than the number of graduates altogether, as not all will work as ATs or remain in Utah.
[33] The National Athletic Trainers' Association (NATA) estimates that approximately half of Utah's secondary schools do not have access to a licensed AT. However, data is based on self-reporting from athletic trainers within each public school, so 51% may be an overestimate due to possible non-response. See: AT Your Own Risk State Statistics Map
[34] OPLR Stakeholder Interviews
[35] OPLR Stakeholder Interviews
[36] Joint Statement from the AT Strategic Alliance
[37] Estimates are based only on the tuition and fee schedules for 2025-26 for BYU, Weber State, University of Utah, and Utah Tech. BYU and Weber state did not give overall program costs, so OPLR assumed two years for BYU's MAT and used Weber State's Master of Science in Athletic Training Graduation MAP to estimate the number of credits per semester.
[38] Utah Department of Workforce Services Economic Data Viewer: Athletic Trainers. Given the limited information DWS has regarding ATs, this estimate could be inaccurate
[39] U.S. Bureau of Labor Statistics Occupational Outlook Handbook: Athletic Trainers
[40] BLS reports that the median salary for individuals working full-time with a master's degree is $1840 per week. OPLR multiplied this by 50 for an estimate of annual salary. See: BLS (2025) Education Pays

However, OPLR does recommend adjusting provisions regarding physician direction and AT scope of practice to increase clarity and give ATs more flexibility in practice. These recommendations are outlined in the following section.

Additional Recommendations

Recommendation: Clarify when an AT must collaborate with a physician

OPLR recommends clarifying when an AT must collaborate with a licensed physician. This recognizes AT education and training, allows ATs to work with more flexibility, and may better ensure safety in cases with complex injuries.

Currently, an AT engaged in the rehabilitation and reconditioning of an injury must do so "as directed by established, written athletic training service plans or protocols, or upon the order of a directing physician". OPLR proposes changing this provision to specify that an AT must collaborate with a physician **only** when rehabilitating or reconditioning an athletic injury that is:

1. beyond the AT's scope of practice or expertise,
2. a suspected head or traumatic brain injury, or
3. not responding to treatment, according to a time period as defined in rule

OPLR also recommends requiring that an AT record all instances of collaboration through injury reports, medical records, or progress reports. Documentation is necessary to ensure compliance with these provisions, as ATs must still collaborate with physicians in specific instances to maintain safe practice.

Requiring physician direction on all injury rehabilitation, regardless of severity or type, ignores the extensive education and training that ATs receive during their master's degree. OPLR's recommendation provides ATs with clearer direction on when they must seek physician guidance, allowing them to practice within their capabilities, but requiring them to contact physicians proactively in more difficult cases where AT treatment is not effective. OPLR does not anticipate that this change will present safety issues. ATs must still collaborate with licensed physicians in any case where the injury is outside of the training and expertise of the AT, ensuring ATs will not be allowed legally to treat injuries that they have not been trained to treat. It may even improve safety if ATs are currently working under outdated or vague physician orders. Oregon has adopted this model of physician oversight,[41] and AT regulators in the state expressed no safety concerns with their model.[42]

[41] OAR 331-160-0015
[42] OPLR Stakeholder Interviews

Recommendation: Explicitly allow ATs to treat work-related injuries

OPLR also recommends clarifying the scope of practice of ATs in statute to explicitly allow them to treat injuries incurred in workplace settings by employees in physically demanding occupations. ATs have been safely working in industrial settings for years,[43] and many states allow this practice.[44]

Utah statute is currently vague on this issue. ATs may treat athletic injuries, which are defined as: "(a) an injury sustained by an athlete that affects the individual's participation or performance in sports, games, recreation, or exercise; or (b) a condition that is within the scope of practice of an athletic trainer identified by a directing physician or physical therapist as benefitting from athletic training services."[45] Injuries sustained through occupational activities do not likely fall under the first definition.[46] Additionally, given OPLR's recommendation to remove the provision requiring ATs to seek physician direction for all injury rehabilitation, it is necessary to clarify that workplace injuries are within an AT's scope to treat without first seeking approval by another licensed healthcare provider.

Other Considerations

Apprenticeship

OPLR carefully considered recommending that Utah create an apprenticeship pathway to licensure, similar to Texas. An apprenticeship pathway allows those with only a bachelor's degree, and at least 1800 hours of an apprenticeship, to become licensed. This would help reduce barriers to entry into the field without sacrificing safety, as Texas regulators indicated no difference in safety or the number of complaints between ATs who have completed an apprenticeship versus those who completed a master's degree.[47]

However, adopting this pathway would come with administrative costs. Like Texas, Utah would have to create and administer a Utah athletic training exam, as individuals with only a bachelor's degree cannot take the BOC exam. There must also be demand for this change. At least one Utah school must be willing to create an unaccredited BA-level AT program, and students must be willing to sacrifice the portability of their license for this alternative route. Although this model works in Texas, it may not be feasible in a smaller state like Utah that likely has less aggregate demand for ATs.[48]

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[43] NATA began surveying companies that utilize athletic trainers as far back as 2003. See: Halls (2009) Executive Summary: Athletic Trainers Provide Return on Investment and Decreased Injuries in Occupational Work Settings
[44] See *Approaches in Other Jurisdictions* for more information on state approaches to AT scope of practice
[45] UCA 58-40a-102(3)
[46] OPLR can not definitively say whether this is true. Conversations with DOPL suggest that no complaint has yet required them to interpret these provisions to either include or exclude injuries from occupational activities.
[47] OPLR Interview Series
[48] Texas likely has greater demand for ATs than Utah due to cultural and population factors; Texas has a much greater population than Utah, far more colleges and programs, and emphasizes high-school sports in a way that Utah does not. Additionally, NATA estimates that only 25% of secondary schools in Texas do not have access to an athletic trainer compared to 51% in Utah. See: AT Your Own Risk State Statistics Map

Still, OPLR suggests that regulators in Utah consider formally adopting this alternative pathway if any university or college expresses interest in creating an unaccredited AT program, or if access to licensed ATs becomes materially constrained. This would require a change in statute to retain a requirement for emergency cardiac certification while not requiring BOC certification in favor of alternative pathways.[49]

Rule Review

In accordance with Utah Code 13-1b-203(5), OPLR conducted an in-depth review of DOPL's athletic training rules, found in R156-40a.

The rule review covered potential rule changes needed to:

1. address specific rules that may be overly burdensome (e.g., for individuals seeking to practice a profession or given the potential risk to public safety from a profession, etc) or insufficient (e.g., to ensure safe practice);
2. address rules misaligned with statutory language, clarify language, and correct references to statute or other rules; or
3. support OPLR's recommendations

OPLR's review of R156-40a found:

1. There is one rule that may be overly burdensome for ATs:
    a. DOPL recognizes CAATE and the BOC as the only organizations that may set curriculum standards for education. This is appropriate as CAATE is the only active AT accreditor and the BOC administers the only national exam, but OPLR recommends allowing for the possibility of accepting other accreditors or certifying/testing bodies should they be created to avoid a monopoly by statute. OPLR is pursuing this recommendation globally for all occupations in this review.
2. There are two incorrect and/or outdated references:
    a. The reference to R156-1-107 in R156-40a-105 is incorrect as R156-1-107 does not exist
    b. R156-40a-502 refers to the 2006 BOC Standards of Professional Practice, an outdated document, for unprofessional conduct
3. New rules regarding the amount of time that would qualify an injury as non-responsive to treatment must be written

[49] BOC certification requires graduation from a CAATE accredited program, passing the BOC exam, and receiving emergency cardiac certification. Removing the requirement to be BOC certified would remove the emergency cardiac certification requirement, which OPLR contends should be maintained regardless of what education or testing requirements the AT applicant meets.

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1. Context

1.1 General Research Methodology

OPLR's methodology combines qualitative and quantitative methods with robust stakeholder engagement. Methods include:

- Conducting quantitative analysis of DOPL licensee and complaint data and publicly available data from other state and federal government entities (e.g., DWS, BLS, NPDB, BOC, NATA)
- Reviewing academic literature and reports on a profession's practice, efficacy and safety
- Scanning education and credentialing requirements, programs and content
- Reviewing state occupational regulation policies across the U.S.
- Engaging with a wide range of stakeholders, including: Utah and other state governments and agencies, industry organizations, researchers, practitioners, and employers

1.2 Athletic Trainer Policy Scan

To better understand the regulatory environment for athletic training, OPLR conducted a review of state occupational regulation in the U.S., which made use of the deep research function of Gemini 2.5 Pro (Google's AI tool). OPLR then validated the Gemini information by verifying Gemini's findings with each state's statute and administrative rule. The validated sources were then used to map the national policy landscape, find patterns in regulation, make cross-state comparisons, and discover outliers. OPLR also used the data to help inform recommendations.

OPLR used Gemini primarily to determine how jurisdictions generally regulate athletic trainers and how they differ regarding specific policies, such as physician collaboration and scope of practice. The following is an example of a prompt used to determine whether states allowed ATs to treat a broader population than athletes:

"Please answer the following question about the regulation of athletic trainers in the U.S. What states have provisions allowing licensed athletic trainers to treat more than just athletes? For example, some states may have removed the reference to athletes or they may have specific requirements for athletic trainers working in other medical or industrial settings. Use only state websites, state administrative code, state administrative rules, and national industry organization websites. Check both the state's practice act (statute) and administrative code/rules. Do not use Reddit, blogs, or other non-verifiable sources. Give me answers in a chart format, with the following columns: state, findings, and full URL link. Only provide me with the chart, I do not need any prose. Ensure you include every state."

For this specific example, Gemini was generally correct in identifying how athlete and athletic injuries were defined, and OPLR manually validated each entry and corrected or added necessary information that Gemini left out.

This review does contain limitations related to normal human error. It is possible that there is slight misreporting of some data due to limited accessible state information, errors in data entry, or mistakes made by Gemini that were not caught by OPLR's manual verification. Misinterpretations of statute or rule are possibilities as well, especially in this review. State law and rule regarding issues such as physician collaboration and AT scope of practice were highly variable across states, often vague, and could not provide specific insight into how each regulatory agency interprets their law or rule. Potential bias or measurement error introduced by these limitations likely did not alter any information substantially, as the Gemini-assisted review was utilized to find patterns rather than report exact numbers.

2. Regulatory Model Assessment and Recommendations

2.1 Menu of Regulatory Models and Factors Considered in Framework

Please see **this working document**, OPLR's Occupational Regulation Framework, for a more detailed explanation of OPLR's approach to assessing occupational regulation and evaluating different regulatory models.[50]

2.2 Model Assessment of Athletic Training

The following tables summarize OPLR's analysis of athletic trainers according to factors that OPLR determined should influence the appropriate regulatory model for an occupation. Factors that OPLR considered particularly determinative in its assessment are highlighted in bold.

Model Assessment of Athletic Training	
Harm Factors	
Mechanism of Harm	**Make an inaccurate evaluation, implement incorrect return-to-play protocols, exacerbate potentially life-threatening injury, cause musculoskeletal injury**
Severity, Permanence, and Likelihood of Harm	Small likelihood of high severity, permanent harm. Greater likelihood of less-severe, temporary harm
Consequence of Error	72*
Downstream Impact	**High. May require immediate or further medical intervention**

[50] The document is also available on OPLR's website in the "About OPLR" section, accessible here: https://oplr.utah.gov/about-oplr/

Consumer & Setting Factors	
Patient Vulnerability	**Moderate. ATs work with those vulnerable due to age (i.e. minors), but the patient population is physically active and typically very healthy.**
Frequency of Physical Touch	**Athletic training requires frequent physical touch, similar to physical therapy**
Frequency of Private Setting	ATs most frequently work in shared spaces with many others around, although they may occasionally treat an individual one-on-one. This is more likely when rehabilitating an injury.
Information Asymmetry	There are high levels of information asymmetry. An individual is unlikely to know the proper protocols for athletic training or what treatment entails. Additionally, injury rehabilitation may cause some low-level pain, so individuals could be harmed but believe it to be a natural consequence.
Related factors	
Level of Independence	*Structural:* Low. Few, if any, ATs own their own businesses or clinics. They are generally always employees. *Clinical:* Moderate. ATs must work in collaboration with physicians, including protocols for assessment and rehabilitation. However ATs must depend on their own clinical judgement when responding to emergencies, making calls to pull out athletes during a game, and adjusting treatment plans for each athlete.
Patient Choice	**Low. Individuals rarely choose their ATs.**
Information Availability	Low. ATs are often employees of larger organizations, which may or may not make information about employees public.
Level of Oversight	*Employers:* Moderate to high. ATs almost always work as employees of existing organizations. *Government:* Outside of licensure, tort or criminal law are the only avenues. Low levels of reimbursement mean low payor oversight. *Private Bodies:* Moderate. The BOC requires continuous certification, disciplines certification holders and can strip individuals of their certification. However, ATs are not required to hold private certification and the enforcement powers of the BOC do not match those of states.

* O*Net Consequence of Error Ranking

Regulatory Model Adjustments & Recommendations

3.1 Possible Adjustments

Please see **this working document**, OPLR's Occupational Regulation Framework, for a more detailed explanation of how OPLR approaches whether adjustments should be made within a recommended regulatory model.[51]

3.2 DOPL Complaint Data

The Division of Professional Licensing (DOPL) receives complaints from individuals, other state agencies, co-workers, professional associations, and licensing boards. DOPL is required to "investigate unlicensed practice in regulated professions, acts or practices inconsistent with recognized standards of conduct, allegations of gross negligence or incompetence, and patterns of gross negligence or incompetence".[52] Violations that meet the criteria for investigation are then prioritized and assigned to an investigator. DOPL may resolve investigations in a variety of ways, including: closing an investigation due to a lack of evidence; referring the case to another agency or to law enforcement if appropriate; carrying out informal or formal administrative sanctions or stipulated agreements; issuing a citation; or denying, suspending, or revoking an individual's license.

To analyze complaints sent to DOPL, OPLR used My License Office (MLO) to access closed complaints investigated by DOPL between 2017-2022 for all licenses/professions. This data contains information on the license name, the complaint type, and the disposition of the complaint, among many other data fields not relevant to OPLR's analysis. DOPL personnel helped code the complaint dispositions as either substantiated, unsubstantiated, or no jurisdiction. Substantiated complaints are those where a disposition includes some type of disciplinary action, whether formal or informal (e.g., letter of concern, verbal warning, surrender of license). Unsubstantiated complaints have dispositions without a disciplinary action (e.g., dismissed, lack of evidence, unfounded). 'No jurisdiction' complaints are complaints that may or may not have basis, but DOPL was not able to take action on the case.

OPLR filtered complaints to exclude any likely duplicates and then used substantiated complaints to calculate the number of complaints per license type or profession. OPLR estimated the complaint rate for each license type by dividing the number of substantiated complaints by the number of unique individuals who held that license type over the same period.

Complaint Case Notes Analysis

[51] The document is also available on OPLR's website in the "About OPLR" section, accessible here: https://oplr.utah.gov/about-oplr/

[52] DOPL, An Explanation of the Complaint Handling Process for the Division of Occupational and Professional Licensing

A more detailed analysis of historical case notes was conducted on the 7 substantiated athletic trainer complaints, and one “no jurisdiction” complaint, closed between 2017-2022.[53] OPLR analysts read through case notes from all eight complaints, summarized the issues, noted whether or not client harm occurred or potentially occurred, and (if harm was present), the type and severity.

Limitations

There are significant limitations to this analysis, and the information collected should not be interpreted as a precise estimate of harm caused by athletic trainers. DOPL data likely underestimates true harm, as many instances of harm may be handled in other ways (e.g., directly by employers), reported to other entities, or may never be reported. Additionally, some unsubstantiated complaints may have resulted in harm but the necessary evidence was not produced.

There could also be latent factors correlated with both the likelihood of complaint and the profession, systematically biasing the comparisons across professions. This is especially true in healthcare, as certain professions, by their nature, include a greater potential for harm and may generate more complaints. For example, surgeons have a higher likelihood of causing severe harm to a patient than ATs because surgery is inherently far riskier, not because surgeons are “less safe” or less competent than ATs.

For these reasons, OPLR uses DOPL complaint data as directionally informative, but avoids direct comparisons across professions wherever possible. Fine comparisons across professions are unwarranted and unsupported by these data.

3.3 DOPL Licensee Data

OPLR used DOPL licensee data queried in January of 2025 to conduct analyses on the number of licensees per year, inflow and outflow of licensees, overlap of licenses, and time with license. The dataset included individuals first licensed after 1970 to those actively licensed as of January 2025. Each row in this dataset was a unique combination of individual and license type and contained information regarding when the license was issued, the status of the license, the date the status was last updated, and the sex and year of birth of the individual. OPLR estimated the number of licensees in each year by summing the number of unique individuals whose licenses were active during any point in each year. Additionally, OPLR excluded any individual with a null or incorrect value for their license issue date and license expiration date, as OPLR could not determine how long or for what years they were actively licensed. License counts may slightly underestimate the true number of licensees due to this, but the effect is fairly negligible given OPLR’s use of the data to determine trends over time rather than estimate with precision for specific dates.

[53] No jurisdiction complaints were included in the case note analysis and not the complaint rate analysis because they are not complaints where DOPL took an action, but they may include legitimate client harm that DOPL had to refer to another agency. Although they can not be classified as “substantiated”, OPLR felt these complaints could help contextualize client harm resulting from PTs and PTAs.

4. Stakeholder Outreach

4.1 OPLR Interview Series

OPLR relied heavily on stakeholder engagement and qualitative interview data, combined with OPLR's other analysis, to conduct this review and develop recommendations. OPLR engaged with ATs, AT employers, athletic training educators, industry associations, Utah legislators, and Utah and other state regulators. OPLR prioritized diversity of perspective and relevance to the industry in selecting stakeholders. Variety in practice setting was also prioritized, and OPLR contacted stakeholders associated with traditional and non-traditional settings (OPLR was not able to contact anyone in the industrial/occupational space however).

Interviews were conducted in person, over the phone, and via video conferencing using semi-structured interview methods; they were conducted one-on-one and with multiple members. Extensive notes were taken for all interviews.

OPLR conducted initial interviews to understand the athletic training industry, determine the largest issues related to safety and access, and identify potential areas for change. OPLR engaged with stakeholders later in its review to test initial findings from analysis and preliminary recommendations. OPLR reflected on and synthesized feedback to develop clear and achievable evidence-based recommendations.

Limitations

This interview sample was not randomly selected and, therefore, is not completely representative. OPLR spoke to individuals most likely to represent the broad aims and concerns of their groups. Additionally, OPLR did not contact "consumers" of athletic training services (i.e. athletes or patients), so their perspectives were not incorporated into this review. Thus, the stakeholder engagement and findings from these interviews should not be understood as fully representative of the views of all Utahns, ATs, AT employers, or any other person, group, or population.

Note that stakeholders' views are not always reflected in OPLR's recommendations. OPLR is directed by Utah Code 13-1b-302 to apply specific review criteria. These can and do lead to recommendations that diverge from stakeholder preferences. A stakeholder's appearance here is not an endorsement of OPLR's recommendations as such.

4.2 OPLR Stakeholder Engagement Summary

Stakeholder Engagement Summary - Athletic Training	
Government Stakeholders	
Utah Department of Commerce	**Margaret Busse**, Executive Director **Carolyn Dennis**, Deputy Director **Jacob Hart**, Deputy Director **Mark Steinagel**, Director, Division of Professional Licensing **Lisa Martin**, Bureau Manager, Division of Professional Licensing
Division of Professional Licensing (DOPL) Board	**Brett Mortensen**, Board Chair, Athletic Trainers Board
Utah State Board of Education	**Kendra Muir,** School Nursing Wellness Specialist, Student Services
Other State Governmental Agencies	**Brian Tobias,** Director of the Colorado Office of Policy, Research and Regulatory Reform **Anne Thompson**, Policy Analyst, Oregon Health Authority Health Licensing Office **Britney Webb**, Presiding Officer, Texas Advisory Board of Athletic Trainers
Industry Stakeholders	
Industry Associations	**Marcus Homer**, President, UATA **Craig Switzler**, Vice President, UATA **Lisa Walker**, Past President, Current Board Member, UATA **Patty Fahringer**, Board Member, UATA
Employers	**Ted Hugunin**, Director of Sports Medicine, Intermountain Health

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